Aflibercept + FOLFIRI

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Aflibercept + FOLFIRI: active treatment cycle

Active medicines: Aflibercept + Irinotecan + Folinic acid + Fluorouracil

Emetogenic risk: Moderate

Medicine	Dose / route	When
Ondansetron	16 mg orally once	Before Day 1 chemotherapy Review QT risk, interactions, constipation and hepatic function.
Dexamethasone	8 mg orally once	Before Day 1 chemotherapy Reconcile all steroid prescriptions; do not duplicate.
Dexamethasone	8 mg orally once daily	Days 2 and 3 after Day 1 chemotherapy Confirm the prescribed delayed prophylaxis and re-anchor it if treatment is delayed.
Metoclopramide	10 mg orally up to three times daily as required	Rescue only; duration to be completed by prescriber Review renal function, neurological contraindications and interactions.
Atropine sulphate	250 micrograms subcutaneously	Before irinotecan if prescribed Avoid in glaucoma. Acute cholinergic support is separate from delayed-diarrhoea treatment.

Additional phase-specific support

- Irinotecan has a moderate risk of sickness without prevention. Follow the prescribed prevention and rescue medicines. Call if vomiting prevents drinking; do not duplicate steroids or anti-sickness doses.
- Growth-factor injections depend on your risk, blood counts and previous complications. If prescribed, confirm their timing in relation to chemotherapy and pump removal; do not assume everyone receives them.
- Atropine is normally prescribed before irinotecan for early cramps, sweating or diarrhoea. It is avoided with glaucoma; tell the team about asthma as well. At home use the separately prescribed delayed-diarrhoea instructions. This can differ from standard over-the-counter dosing. Keep the medicine and urgent-contact plan available.
- National Cancer Control Programme (NCCP) delayed-diarrhoea rescue: loperamide 4 mg initially, then 2 mg every 2 hours until 12 hours after the last liquid stool; do not continue this high-dose schedule beyond 48 hours. Provide urgent escalation for fever, dehydration, pain or persistent diarrhoea. This is not the capecitabine-only rescue schedule.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00238 v6 · NCCP NCIS antiemetics V8, Table 1; apply oral regimen guidance separately

Sources checked 2026-11-04 · SACTCheck v0.84.33 · Regimen-level drafts; prescriber and oncology-pharmacy verification required